APPLICATION · HEAD OF FUNDRAISING & GRANTS

Funding Floaid

A 24-month capital plan for FLOCATH and Floex 3.0 — equity, grants, debt and CSR

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Prepared 28 August 2026 · Built from the job description, floaid.com, CDSCO notified-device schedules, DoP / BIRAC / TDB / ANRF scheme documents and 2026 funding data. Every figure is sourced; unverified items are flagged.

HOW I READ THE MANDATE

Four different risks. Four different kinds of money.

GLP animal studies, first-in-human, a CDSCO Class D licence and a GMP plant do not price the same way — and should not be funded from the same pocket.

GLP animal studies + first-in-human

Pure technical risk.
No equity investor prices this well.

GRANT MONEY

CDSCO Class D licence + clinical pathway

Time risk, not capital risk.
Needs people and process.

EQUITY + STATE SUPPORT

GMP facility, SIPCOT Oragadam

Capex against a known asset. Bankable.

SUBSIDY + SOFT DEBT

Commercial launch, Floex 3.0

Working capital against orders, not promises.

REVENUE + DEBT

The consequence for the ask

If the grant and soft-debt tracks are run properly, the equity round shrinks. India's RDI Fund lends at a 3% spread over 12–15 years to TRL-4+ medical-device companies, and the Department of Pharmaceuticals reimburses 25% of animal-study and clinical-investigation cost. Money that would otherwise be bought with 20–25% of the company. That is the single largest value this role creates — not the round itself, but how much of the company survives it.

Sources: ANRF RDI Fund (rdifund.anrf.gov.in); TDB RDI-SLFM terms; DoP Scheme for Strengthening of Medical Device Industry, operational guidelines 08.11.2024.

DIAGNOSIS

Six problems inside this job description

The JD describes activities. These are the problems underneath them — and where I would start.

1 Two products, two capital classes

FLOCATH is Class D. Floex 3.0's console is Class B. They cannot share a funding narrative or a licence timeline.

2 The non-dilutive calendar has already turned

BIRAC-RDIF Call 1 closed March 2026. SMDI's window closed 23 July 2026. SMDI itself lapses in FY2026-27 — perhaps one cycle left.

3 Half the international list is a dead end

Wellcome excludes India and for-profits. NIH abolished foreign subawards in May 2025. Gates is infectious disease. Saying so early saves a quarter.

4 The entity may not match the incentives

A Delhi CIN with Tamil Nadu operations sits awkwardly against TN schemes and the RDI Fund's residency test. Fix before applying, not during.

5 The market number does not exist yet

Public Indian PCI data stops at 438,351 (2018). A bottom-up high-risk PCI / cardiogenic shock TAM has to be built, not cited.

6 CSR to your own foundation has a trap

Schedule VII clause (ix) requires government funding. Third-party donors are blocked for three years under Rule 4(1)(d). There is a way through.

Each of the six is addressed with a named instrument, a named counterparty and a date later in this deck.

THE MARKET — I

India buys its advanced cardiac hardware. That is the investment case.

Sector data: EEPC India / Invest India medical device sector overview; IBEF industry brief 2026.

US$50 bn

Projected Indian medical device market by 2030 (US$15.4 bn, 2023)

75–80%

Import dependency — essentially unmoved after a decade

24%

Share of the Indian market held by "other medical devices", the class FLOCATH and Floex sit in

₹3,420 cr

PLI outlay whose named segments include cardio-respiratory devices and implants

What the import table actually says

Needles, catheters and cannulae are simultaneously India's #1 medical device export and its #1 import — India makes the commodity and buys the complex version of the same product.

Therapeutic respiration and artificial-organ apparatus sits in the top ten imports. Heart-lung machines and circulatory support are in that line.

The honest version

Sophisticated healthcare investors have seen the "$50 bn by 2030" slide too often. The credible counter-view puts India at US$20–25 bn, and notes that Indian MedTech returns have accrued to distribution and service annuities, not manufacturing.

Floaid's answer: a pVAD is a high-value, single-use, clinically differentiated consumable — the one category where manufacturing margin survives. I would put that objection on the slide rather than wait for it in the room.

Sources: EEPC India, "Indian Medical Device Sector" (segment shares, trade table); IBEF Medical Devices, 2026; InsightRx, "MedTech market India 2026"; Department of Pharmaceuticals PLI scheme.

The pVAD category is the fastest-growing line inside J&J MedTech

And the incumbent has been legally available in India for nineteen years with negligible penetration. That is the question to answer, not avoid.

$8.93 bn

J&J MedTech cardiovascular revenue, 2025 — up 15.8%, the only J&J unit growing double digits

> $1 bn

Abiomed / Impella franchise revenue, 2025

$16.6 bn

What J&J paid for Abiomed

The Indian arithmetic that matters

India PCI volume, 2018	438,351
PCI volume, 2011 (7× growth in a decade)	~150,000
PM-JAY cardiac procedures, FY19–FY22	242,580
PM-JAY single-stent PCI package rate	₹72,200
Impella, all-in India procedure cost	US$3,500–5,500

The objection I would prepare for first

"Impella has been DCGI-registered in India since March 2007 and still has negligible penetration. Why is price the binding constraint rather than cath-lab readiness, training and reimbursement?" A crisp, evidenced answer to this decides the round.

The market-creation argument

A ₹72,200 public package buys an entire single-stent PCI. An Impella procedure costs roughly five times that. A pVAD at one-sixth of Impella is the first such device that could plausibly sit inside — or adjacent to — a public cardiac package.

That, not the global TAM, is the Indian argument. It is also why the affordability story has to be evidenced against reimbursement, not against a list price.

Sources: MedTech Dive (J&J FY2025 results); J&J press release, Abiomed acquisition; EPW, "PCI under Ayushman Bharat" (NIC data); BioSpace (DCGI Impella registration, 29 Mar 2007); Medigence (India procedure cost — aggregator, directional only).

Floex 3.0 is the sober half — and the half that can carry debt

Five global incumbents. No indigenous Indian heart-lung machine manufacturer that I could verify. An installed base that already buys service contracts.

Who owns the category today

- **Getinge**
- **LivaNova**
- **Terumo Cardiovascular**
- **Medtronic**
- **Nipro**

Each is also a plausible strategic investor or distribution partner for Floex 3.0 — which is a second reason to build the relationship now.

Why this is the fundraising lead, not the headline

Floex 3.0 approaching commercial launch gives the raise three things FLOCATH cannot yet: revenue that a lender can underwrite, a perfusionist and cardiac-surgery relationship inside the hospital, and a shorter regulatory path.

I would lead investor conversations with Floex revenue credibility and sell FLOCATH as the asymmetric upside — the reverse of how most device decks are built, and the reason most of them stall.

Import substitution is the policy hook — and it is worth real money

The Department of Pharmaceuticals runs a scheme literally called the **Marginal Investment Scheme for Reducing Import Dependence** — a one-time capital subsidy of 20% of investment, capped at ₹10 crore, for companies under ₹250 crore turnover manufacturing finished devices, key components or accessories. An indigenous heart-lung machine and an indigenous catheter pump are exactly the products the scheme was drafted to create. The Oragadam plant is the eligible investment. **This is the single best-matched capital instrument for the facility, and it does not appear in the JD.**

Sources: DoP SMDI operational guidelines (08.11.2024), Component 2; cardiopulmonary bypass pump market reports; Octopus Med / HeartCare India (Indian HLM supply is distribution-led).

THE INSIGHT I WOULD BUILD THE PLAN ON

Two products, two CDSCO risk classes, two capital tracks

From the CDSCO notified medical device schedules — Cardiovascular (26 July 2021) and Catheters.

FLOCATH · percutaneous VAD

CDSCO entry	"Ventricular bypass (assist) device"
Risk class	Class D — highest risk
Licence route	Central Licensing Authority (MD-9)
Clinical	Clinical investigation required
Comparable	"Intra-aortic balloon and control system" is also Class D

Funded by: grants + equity. No lender underwrites a pre-first-in-human Class D device.

Floex 3.0 · heart-lung machine

CDSCO entry	"CPB heart-lung machine console"
Risk class	Class B — low-moderate risk
Licence route	State Licensing Authority (MD-5)
Circuit parts	Mostly Class B/C; pulsatile flow generator is Class D
Adjacency	ECMO circuit for long-term support is Class C

Funded by: capital subsidy + soft debt + revenue. A Class B console is a bankable manufacturing asset.

Why this matters to a funder

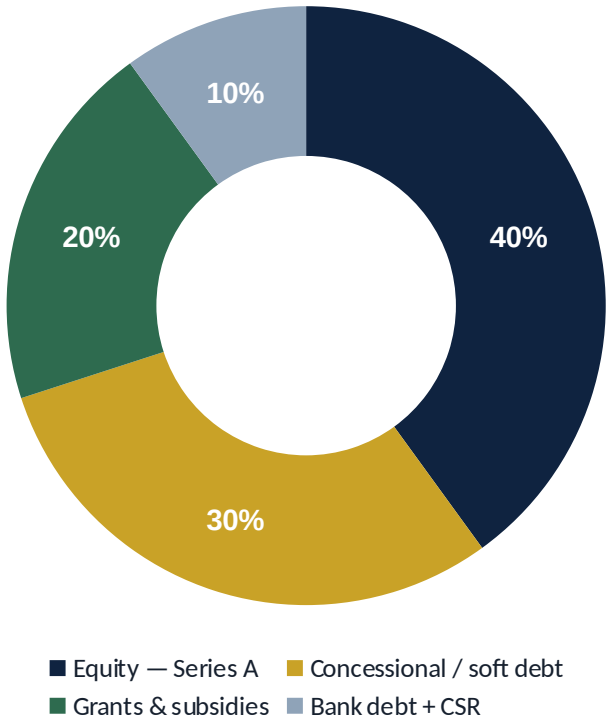
A single blended "MedTech" story asks one investor to price two very different risks and gets punished on both. Separating them lets the Oragadam facility be financed largely against Floex 3.0 — subsidy, SIDBI STHAPAN and a guaranteed bank facility — while equity and grant money are reserved for the Class D clinical programme. Same company, two balance-sheet conversations.

Source: Notified Medical Devices schedules under the Medical Devices Rules 2017 — Cardiovascular (notified 26.07.2021) and Catheters lists, as compiled in the CDSCO notified-devices e-book, Edition 04.

THE PLAN · 24 MONTHS

One architecture, sequenced so the cheapest money is raised first

Illustrative structure at a ₹150–200 crore total programme cost. Real numbers follow from the model and the board plan.



Instrument	Named source	Scale	Cost of capital
Concessional debt	ANRF RDI Fund via TDB / BIRAC	50% of project cost	3% spread, 12–15 yrs
Clinical grant	DoP SMDI — Clinical Studies Support	₹2 cr animal + ₹5 cr clinical	Free (25% reimbursement)
Capital subsidy	DoP SMDI — Marginal Investment	20%, up to ₹10 cr	Free
State subsidy	TN MSME thrust-sector capital subsidy	25%, up to ₹1.5 cr	Free
Plant debt	SIDBI STHAPAN + MCGS-MSME guarantee	Up to ₹20 cr, 75% of cost	6.0–7.3%, 7 yrs
Equity	Series A syndicate, 2–4 funds	₹55–70 cr	18–25% dilution
R&D refund	Australian R&DTI via CardioBionic	43.5% of qualifying spend	Free (partner-routed)

Sequencing rule: every rupee of grant or 3% debt secured before the term sheet is a rupee of equity not sold. The grant applications are therefore not a parallel workstream — they are on the critical path to the valuation.

Percentages illustrate structure, not a committed plan. Instrument terms are sourced individually on the slides that follow.

VENTURE & STRATEGIC CAPITAL · ~50% OF THE ROLE

How I would design the Series A process

Nine weeks from mandate to first partner meeting; a single tightly-run process rather than a rolling conversation.

Weeks 1–3	**Materials & data room**	Rebuild the deck around the two-track story. Five-year model with the grant and soft-debt tranches modelled explicitly. Cap table with three dilution scenarios. Use-of-funds tied to GLP → FIH → MD-9 milestones. Virtual data room structured to CDSCO and ISO 13485 diligence, not generic SaaS headings.
Weeks 4–5	**Target list & sequencing**	60–80 names scored on cheque size, device thesis evidence, fund vintage and warm-path strength. Four waves. Warm paths mapped through IITM Research Park, ARTPARK, MGM Healthcare and the existing cap table before a single cold email.
Weeks 6–9	**Outreach & first meetings**	Wave 1 approached in one block so the process has a clock. Weekly investor-pipeline review with the CEO. Every "no" logged with a reason, and the reason fed back into the materials.
Weeks 10–20	**Term sheet to close**	Term-sheet negotiation support, confirmatory diligence coordination, legal and finance closing checklist, and the shareholder communication that goes out the day it signs.

The JD asks for a live pipeline of 50+ qualified contacts within twelve months. The list on the next slide is 85 names, already scored. That part of the job is done.

THE TARGET LIST

Four waves — warm and local first, strategics last

85 names scored in the accompanying workbook. These are the ones I would move on in month one.

WAVE 1

Warm, local, actively deploying

IITM Unicorn Frontier Fund I

₹600 cr, launched Feb 2026, MedTech named, same research park, 40% follow-on reserve

Inflexor Ventures

₹15–45 cr; Fund III first close Jul 2026; led Ayati Devices MedTech round Aug 2026

Endiya Partners

₹25–50 cr; leads pre-Series A; IFC and AIIB backed

Speciale Invest

Chennai; Growth Fund II $5–8M, "health and biosciences" named

ANRF RDI Fund via BIRAC

Run in parallel, not after — it changes the size of the equity ask

WAVE 2

Dedicated healthcare capital

Kotak Life Sciences Fund

₹40–65 cr cheques written in 2026

HealthQuad (Quadria)

₹550 cr first close Jun 2026; MedTech in the stated thesis

360 ONE Asset

₹500 cr fund; healthcare + precision manufacturing + deeptech

pi Ventures · Anicut (Chennai)

Science-literate seed; ₹5–8 cr local co-invest slice

WAVE 3

Strategics — validation and channel

Terumo Ventures

$75M cardiovascular CVC; Terumo already sells perfusion systems

Trivitron · Meril · SMT

Chennai HQ; cath-lab salesforce; F-Prime backed

Siemens Healthineers

Already co-funds the ARTPARK/IHFC MedTech grant — a warm door

Getinge · LivaNova

The Floex 3.0 route

WAVE 4

Series B relationships to seed now

Somerset Indus · Quadria

Tier-2/3 affordability thesis maps onto a 1/6th-price pVAD

F-Prime · Eight Roads

F-Prime holds Trivitron — the only global life-science VC verified with an Indian device manufacturer

LeapFrog · BII · IFC

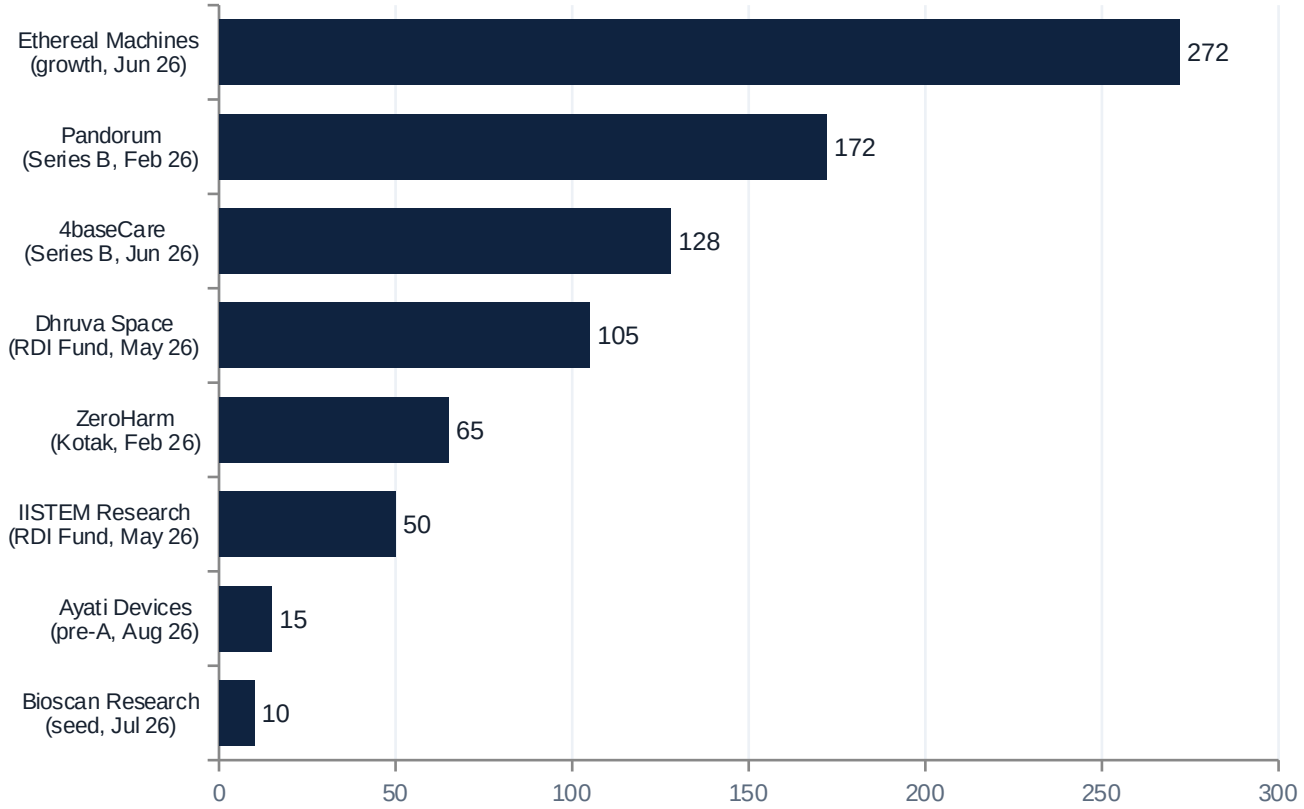
Development capital; affordability mandate

All fund facts verified against primary or reputable secondary sources dated 2025–26 and listed in the accompanying workbook. Funds whose device thesis could not be verified are flagged there rather than presented as targets.

CALIBRATION

What an Indian MedTech Series A actually looks like in 2026

Bounded by the cheque sizes the active funds have publicly committed to — not by a global benchmark.



Realistic parameters

Round size	₹40–80 cr, syndicated
Lead cheque	₹20–45 cr
Pre-money, pre-FIH	₹150–300 cr
Dilution	18–25% + ESOP top-up
Instrument	CCPS, 1× non-participating
Tranching	Milestone-linked, near universal

The ask I would take to market

₹55–70 crore of equity alongside ₹40–100 crore of RDI Fund concessional debt at a 3% spread for the GMP facility.

The blended structure is the differentiated ask. It funds a capex-heavy SIPCOT build without selling 40% of the company — and no competing device deck in the market is arriving with it already structured.

Sources: Indian Startup News, Entrackr, Inc42, YourStory, BW Disrupt, Businesswire (individual round reports, 2026); RDI Fund first disbursements, 13 May 2026. Parameter ranges are inferred from these, not a published statistic.

DILIGENCE

Three things I would fix before the deck goes out

Every one of these will surface in diligence. Each is cheaper to fix now than to answer live.

1

The entity does not match the incentives

The public registry shows FLOAID MEDTECH PRIVATE LIMITED with a Delhi CIN (2024), while operations sit at IIT Madras Research Park and the plant is planned for SIPCOT Oragadam.

Fix: Tamil Nadu's schemes and the RDI Fund's "registered in India and controlled by resident Indian citizens" test both look at the entity. Resolve domicile, or structure a TN subsidiary, before any application is filed.

2

The market number does not exist yet

Public Indian PCI data stops at 438,351 procedures (2018). There is no authoritative national figure for cardiogenic-shock incidence.

Fix: Commission an updated NIC/CSI volume figure and a clinician-panel estimate of the addressable high-risk PCI fraction. Investors will not accept a top-down global TAM for a category with near-zero Indian penetration.

3

The public story and the pitch do not agree

The website team page names Dr K Balakrishnan (not "KR"), does not mention ArtPark, and discloses no investor. The JD, the site and the deck should say the same thing.

Fix: A thirty-minute fix that removes an unnecessary diligence question. The same discipline applies to the patent count — 9 filed and 15 in pipeline needs a docket, not a claim.

Registry detail from public corporate-registry aggregation; team detail from floaid.com/team as at 28 August 2026.

GRANTS & NON-DILUTIVE · ~35% OF THE ROLE

The Indian ladder, ranked by what it is actually worth to Floaid

The JD lists schemes. This is which of them pay, which have closed, and which never applied.

Instrument	Ticket	Instrument type	Fit	Window status
ANRF RDI Fund (BIRAC / TDB)	₹5–200 cr	OCPS ≤₹25 cr; unsecured loan >₹50 cr at 2–3%	Excellent — TRL 4+, medical devices named	Call 1 closed 31 Mar 26; Call 2 unannounced
SMDI — Clinical Studies Support	₹2 cr animal + ₹5 cr clinical	25% reimbursement	Excellent — funds GLP and FIH directly	Closed 23 Jul 26; scheme lapses FY27
SMDI — Marginal Investment	20%, cap ₹10 cr	Capital subsidy on reimbursement	Excellent — the Oragadam plant	Closed 23 Jul 26; likely one cycle left
TDB — core + RDI-SLFM	50–70% of project cost	Loan at 5% simple, or RDI at 2–3%, 15-yr tenor	Strong — its own call names Class A–D devices	Rolling + periodic calls
BIRAC i4 — BIPP	Not published	Grant-in-aid, milestone, co-contribution	Good at TRL 7+; needs DSIR R&D recognition first	Annual, ~Apr–May
TN MSME capital subsidy	25%, cap ₹1.5 cr	Subsidy	Good — medical devices is a TN thrust sector	Rolling
CGSS / MCGS-MSME	≤₹20 cr guarantee; 60% cover to ₹100 cr	Credit guarantee	Good — collateral-free GMP capex debt	Rolling
BIG · SISFS · NIDHI PRAYAS · PLI	—	—	Dead ends: stage mismatch, turnover caps, closed windows	Do not spend cycles

The Tamil Nadu Life Sciences Promotion Policy 2022 adds a materially larger package — capital subsidy, 100% stamp-duty exemption on SIPCOT land, R&D personnel support and 50% reimbursement of WHO-GMP/CE certification cost — but only above a ₹50 crore eligible-fixed-asset threshold, and it expires 31 March 2027. Whether the Oragadam facility is scoped above or below ₹50 crore is therefore a capital-strategy decision, not just an engineering one.

Sources: rdifund.anrf.gov.in; tdb.gov.in/rdi_slfmss; DoP SMDI operational guidelines 08.11.2024 and call notices Jun–Jul 2026; birac.nic.in; msmetamilnadu.tn.gov.in; PIB (CGSS expansion, 9 May 2025); TN LSPP 2022.

THE GRANT CALENDAR

What I would file, and when

Applications are not a background task — the windows are narrow, mostly annual, and two of the best have already closed once this year.

Sep – Nov 2026

- Register on the BIRAC-RDIF portal; pre-build the Call 2 dossier against TRL-4 evidence
- Get application-ready on smdi.lsssdc.in for both MDCSSS and MIS-RID; verify the revised clinical caps with DoP
- Enrol with MedTech Mitra (NITI Aayog / ICMR / CDSCO) — no cash, but it de-risks the Class D pathway
- Incorporate the Floaid Research Foundation; file 12A, 80G and CSR-1 to start the three-year clock

Dec 2026 – Feb 2027

- TDB RDI-SLFM approach — a parallel, non-competing shot at the same RDI corpus as BIRAC
- Secure DSIR in-house R&D unit recognition — it gates BIPP
- Guidance TN / SIPCOT meeting: Oragadam land rate, plot availability, Kancheepuram A/B/C classification
- Model the plant at ≥₹50 cr EFA (LSPP 2022) versus the MSME route — LSPP expires 31 Mar 2027

Mar – Aug 2027

- BIRAC i4 / BIPP annual call (~Apr–May)
- SMDI final cycle before the scheme lapses at the end of FY2026-27
- Australian R&DTI Overseas Finding with CardioBionic — must be lodged before 30 June
- Horizon Europe Cluster 1 consortium, 13 Apr 2027 deadline, positioned to convert if India associates

Watch list: BIRAC-RDIF Call 2 · SMDI final cycle · TDB medical devices call round 2 · TANSEED 9.0 · operative guidelines for Tamil Nadu's ₹100 crore deep-tech policy · India–EU Horizon association, targeted for 2027

INTERNATIONAL FUNDING

Honest triage — most of this list is not available to Floaid

Saying so in week one is worth more than a quarter spent discovering it.

CLOSED TO FLOAID

Wellcome Trust

Not-for-profit hosts only, and India is explicitly excluded from the LMIC list. The Innovator Awards, which did fund devices, closed in 2021.

Gates / Grand Challenges

Infectious disease, diagnostics and LMIC delivery. Advanced circulatory support is the opposite of their cost-per-DALY logic. The 2026 India call closed 1 July.

NIH / NIBIB

SBIR is US-only. NIH stopped issuing awards containing foreign subawards on 1 May 2025 (NOT-OD-25-104). A US partner can no longer simply pass money through.

EIC Accelerator

EU/associated-country SMEs only. Applying would require genuinely relocating the company before Step 2.

GENUINELY AVAILABLE

Australian R&D Tax Incentive

Via CardioBionic. 43.5% refundable offset, live today. The concrete mechanism — detailed on the next slide.

Horizon Europe Cluster 1

India cannot be funded yet, but formal association talks opened 17 July 2026 targeting 2027. Join a consortium now as a self-funded partner and convert.

Bilateral: USISTEF, AISRF, IGSTC

₹1.5–3.5 crore scale, genuinely eligible. USISTEF's "Healthy Individual" track names affordable biomedical devices. BIRAC funds the Indian industry side.

Wellcome Leap

Open to companies globally, milestone-based contracts, $45–60M programmes. Nothing open in cardiac now — join the Health Breakthrough Network to be on the list.

There is no cardiovascular-device equivalent of CARB-X. No dedicated international non-dilutive accelerator exists for this category, and the board should hear that plainly rather than see it in a pipeline that never converts.

Sources: wellcome.org eligibility guidance; grants.nih.gov NOT-OD-25-104; eic.ec.europa.eu; research-and-innovation.ec.europa.eu (India association, 17 Jul 2026); BIRAC Grand Challenges India; wellcomeleap.org.

The most actionable international route — and it is a tax instrument

Australia's R&D Tax Incentive pays a 43.5% refundable offset to a loss-making company. An Overseas Finding extends it to work Floaid does in India.

1 The four conditions

The overseas work must be a core or supporting R&D activity; there must be at least one core activity conducted solely in Australia that cannot be completed without it; the work must genuinely be impossible in Australia (cost alone is not a reason); and total overseas cost must be less than the Australian cost.

2 The structure

CardioBionic runs the Australian core activities — bench validation to ISO 5840 / 14708, large-animal work. Floaid runs India-side activities that demonstrably cannot be done in Australia. The commercial agreement channels the benefit back.

3 The trap

The Advance/Overseas Finding must be lodged before the end of the Australian income year in which the overseas work happens — by 30 June, never retrospectively. Average processing is 90 days.

4 What it is worth

Roughly ₹30 lakh of cash refund for every ₹1 crore of qualifying Australian-side spend, at a 43.5% offset. Non-dilutive, repeatable annually, and it survives a tax loss.

43.5%

Refundable offset for an Australian R&D entity under AUD 20M turnover — paid as cash even at a tax loss

Rates hold until 1 July 2028. The 2026-27 Budget then raises the refundable threshold to AUD 50M but removes supporting-activity eligibility — which is where a lot of prototype work is claimed. Plan around that date.

Also worth pursuing

MRFF International Clinical Trial Collaborations — the stream most likely to accommodate an Indian trial site, via an Australian administering institution.

CRC-P Round 20, expected Q1 2027 — CardioBionic leads, Floaid participates in kind.

AISRF Round 17 — BIRAC funds the Indian industry side with no stated ceiling.

Sources: ato.gov.au and business.gov.au (R&DTI rates, overseas activities, Advance/Overseas Finding); Baker McKenzie (2026-27 Budget changes); nhmrc.gov.au; BIRAC Indo-Australian guidelines.

CSR & THE FLOAID RESEARCH FOUNDATION

The JD cites the wrong clause — and the right structure has a clock

Getting this wrong does not fail slowly. It fails at the donor's audit, after the money has been spent.

The clause (ix) trap

Schedule VII clause (ix) covers contributions to incubators and R&D projects — but only those "funded by the Central Government or State Government or a PSU or any agency" thereof. A privately established Floaid Research Foundation does not qualify under it unless the specific project is itself government-funded.

Healthcare is clause (i), not clause (ii). Frame the cardiovascular innovation hub as "promoting health care" — clause (i) carries no government-funding qualifier.

The asymmetry that works in your favour

The three-year track-record requirement sits only in Rule 4(1)(d) — the residual limb for unrelated donors. A foundation established by the company itself falls under Rule 4(1)(a), which carries no such condition.

So Floaid MedTech (or a co-founding company) can fund the Foundation from day one. Third-party corporate donors are the ones blocked for three years. Incorporate now: the clock only starts on incorporation.

The setup checklist

- Incorporate as a Section 8 company (or registered public trust/society)
- Income-tax registration under Section 12A
- Income-tax approval under Section 80G
- File e-Form CSR-1 with the Registrar, digitally verified by a CA/CS
- Schedule VII mapping in writing — clause (i) for the hub, not clause (ix)
- Board opinion from company-secretarial counsel before soliciting anything touching the GMP plant

The 2026 change nobody has priced in

The Companies (CSR Policy) Amendment Rules 2026, effective 27 May 2026, insert Rule 4A: companies may deploy CSR by subscribing to Zero Coupon Zero Principal instruments issued by NPOs listed on a SEBI-recognised Social Stock Exchange — with no CSR-1 requirement and no three-year bar.

SSE registration is therefore an alternative to the waiting period. Capped at 10% of each donor's CSR, so it suits a many-donors model. This deserves its own legal workstream.

Sources: Schedule VII, Companies Act 2013 (G.S.R. 525(E), 24 Aug 2020); Companies (CSR Policy) Rules 2014, Rule 4 as amended 7 Jul 2025; Amendment Rules 2026, Rule 4A. Not legal advice.

DEBT, INCENTIVES & OTHER INSTRUMENTS · ~15% OF THE ROLE

Financing Oragadam without dilution

The honest answer on venture debt first, then the stack that actually works.

Venture debt: not yet, and approaching early burns the relationship

Indian venture debt underwrites the next equity round, not the next clinical milestone. The universal gate is institutional VC backing, 12+ months of runway post-debt and generally post-Series A. Terms run 13–15% with 0.1–2% warrants.

The right move: size a facility at 20–30% of the Series A, and open the conversation only once a term sheet is signed.

Who to talk to, and when

InnoVen Capital	NBFC; the most flexible, seed-stage willing
Trifecta Capital	₹10–15 cr; post-Series A
Alteria Capital	To ₹150 cr; healthcare in mandate
BlackSoil	Structured facilities, NBFC + AIF
Stride Ventures	₹8,350 cr commitments; India + GCC

THE STACK THAT ACTUALLY FUNDS THE PLANT

SIDBI STHAPAN

Up to ₹20 crore, 75% of project cost, 6.00–7.30% in year one, up to 7 years with a 2-year moratorium. Built explicitly for new enterprises buying capital assets. Minimum 25% promoter contribution.

MCGS-MSME guarantee

The functional ECLGS successor. 60% NCGTC cover on loans up to ₹100 crore, provided at least 75% of project cost is equipment — which a GMP plant is. No guarantee fee in the sanction year.

SMDI Marginal Investment

20% capital subsidy, capped ₹10 crore, on reimbursement after commissioning.

TN capital subsidy stack

25% of plant and machinery to ₹1.5 crore (medical devices is a declared thrust sector, so this is statewide), plus a 20% low-tension power tariff subsidy for 36 months and a 3% back-ended interest subsidy.

SIPCOT Oragadam

100% stamp-duty exemption on SIPCOT land; land-cost concession; 5-year electricity tax exemption. Land rate, plot availability and Kancheepuram's A/B/C classification are unpublished — three calls to Guidance TN settle them.

Sources: SIDBI STHAPAN terms; NCGTC MCGS-MSME product page and PIB release; DoP SMDI guidelines; msmetamilnadu.tn.gov.in; TN LSPP 2022; EquityList India venture debt guide, 2026.

EXECUTION

First 90 days

Two things have to be true by day 90: the Series A process has a clock on it, and no non-dilutive window is missed because nobody was watching it.

Days 1–30

Establish the base

- Read every existing investor conversation, term sheet draft and grant file; build the single source of truth
- Resolve the entity-domicile question with counsel before any application is filed
- Rebuild the financial model — five years, with grant and soft-debt tranches modelled as separate lines
- Register on the BIRAC-RDIF and SMDI portals; enrol with MedTech Mitra
- First shareholder update out on a fixed monthly date

Days 31–60

Build the machine

- Investor pipeline live: 60–80 scored names, warm paths mapped, owner and next action on every row
- Data room built to CDSCO and ISO 13485 diligence structure, not a generic index
- Incorporate the Floaid Research Foundation; file 12A, 80G, CSR-1
- Guidance TN and SIPCOT meetings held; Oragadam incentive package quantified
- DSIR in-house R&D recognition application filed

Days 61–90

Go to market

- Wave 1 approached as one block — IITM Unicorn, Inflexor, Endiya, Speciale
- RDI Fund and TDB engaged in parallel, so the equity ask is sized against confirmed soft debt
- Australian R&DTI Overseas Finding scoped with CardioBionic's adviser
- Bottom-up Indian high-risk PCI and cardiogenic-shock TAM commissioned and delivered
- Weekly capital review with the CEO running as a standing meeting

ACCOUNTABILITY

The twelve-month scorecard, against your own success criteria

These are the four outcomes the job description defines. Here is how I would make each one measurable from month one.

Your stated success criterion	How it becomes measurable	Leading indicator I would report monthly
Series A closed on terms aligned with the board's plan	Round size, pre-money, dilution and tranche structure agreed with the board in month one and held as the reference case	Meetings held → second meetings → diligence started → term sheets. Conversion at each step, with the reason logged on every drop-out
At least two non-dilutive awards covering GLP and clinical trial cost	Applications filed, not opportunities identified. Target: six credible filings to land two awards	Applications in draft, filed, under evaluation, awarded — and days to the next window closing on each
A live pipeline of 50+ qualified contacts, monthly updates on schedule	85 names already scored. "Qualified" defined as: verified device or deeptech thesis, cheque size fits, warm path identified	Pipeline count by wave and stage; update sent on a fixed date, no exceptions
Grant compliance and reporting running without exceptions	A compliance calendar per award from the day it is sanctioned: utilisation certificates, milestone reports, audit dates	Zero overdue items. This is a binary metric and should be reported as one

The compensation structure in the JD is partly linked to funds raised. That only works if "raised" counts non-dilutive rupees as well as equity rupees — otherwise the incentive quietly pushes against the cheapest capital available. Worth agreeing before signing.

HOW I WOULD RUN IT

The operating system behind the numbers

A fundraising function is a pipeline, a calendar and a compliance register. Built once, it survives the person running it.

Pipeline

- One workbook, one source of truth: investor, stage, owner, last touch, next action, reason-for-no
- Scored on cheque fit, thesis evidence, fund vintage and warm-path strength
- Reviewed weekly with the CEO, on a standing slot
- Every declined conversation writes back into the materials

Calendar

- Every scheme window, deadline and call cycle on one sheet with days-to-close
- Owner and technical inputs named per application, agreed with engineering and clinical up front
- Nothing is "identified" — it is drafted, filed or explicitly declined
- Quarterly horizon scan for new notifications

Data room

- Structured to how a device investor diligences: regulatory, clinical, IP, quality system, manufacturing, commercial
- Regulatory timeline maintained as a live document, not a slide
- Patent docket kept current — filings, grants, jurisdictions
- Versioned, access-logged, and never rebuilt in a hurry

Compliance

- A register per sanctioned award: milestones, budget heads, utilisation certificates, audit dates
- Statutory auditor engaged early — SMDI disburses on auditor certification and published results
- Reported as a binary: zero overdue items, or an exception with a date
- Shareholder and grant reporting share one monthly rhythm

FIT

Why me

Read against the JD's must-haves, honestly — including where I am not the obvious candidate.

✓ Government funding is what I have actually done

Invest India and MPIDC/GITA: investment promotion and state investment strategy. I have worked the scheme side of the table — which is where two-thirds of this role sits.

✓ Written applications that get read

Scheme applications, incentive packages and investment cases are the same craft: a compliant document that survives an evaluator who has read three hundred of them.

✓ Financial fluency, built not claimed

Five-year models, capital-structure work, subsidy and incentive modelling across energy, chemicals, agri-processing and healthcare. I build the model; I do not brief someone else to.

✓ Credible across disciplines

Chemical engineering at IIT Madras and LNG process engineering at Shell, then an MBA at ESMT Berlin. I can sit with the engineering team on a hemolysis result and with an investor on the same afternoon.

✓ Chennai-based, and the network is here

IIT Madras, the Research Park ecosystem, Guidance TN and SIPCOT. The JD says hybrid is not possible. Good — I live here.

Where I would need help

I have not previously sat inside a MedTech company through a CDSCO Class D pathway. I know the scheme architecture and the capital structure; the clinical and regulatory judgement has to come from Dr Balakrishnan, the engineering team and MedTech Mitra — my job is to translate it, not to originate it.

My existing relationships are strongest in government, industrial and energy capital rather than among healthcare VC partners. That gap closes in a quarter of deliberate work; the government side takes years to build and I already have it.

I would rather say this now than have it emerge in month three.

BEFORE STARTING

What I would want agreed in the first week

Five decisions that shape everything downstream. None of them is mine to make alone.

1 The board's reference case

Round size, pre-money floor, acceptable dilution and whether strategic money is welcome on the cap table. Without this, every investor conversation is improvised.

2 Entity and domicile

Resolved with counsel before the first application. It gates the RDI Fund residency test and the entire Tamil Nadu incentive stack.

3 The scope of the Oragadam facility

Above or below ₹50 crore of eligible fixed assets changes which state package applies and by how much. This is a capital decision with an engineering consequence, not the reverse.

4 Technical input, committed

Grant applications need engineering and clinical time on a schedule. Named contributors and agreed turnaround, or the applications slip and the windows close.

5 How "funds raised" is defined

If the performance-linked component counts only equity, it argues against the cheapest capital in the plan. Counting non-dilutive rupees aligns the incentive with the outcome.

Accompanying this deck: a scored workbook of 85 named investors, the ranked non-dilutive instrument list with windows and sources, and the grant calendar to August 2027.

The argument, in one line

Floaid does not primarily need someone who can raise a Series A. It needs someone who can make the Series A smaller.

India has just built the largest non-dilutive funding architecture in its history — a ₹1 lakh crore RDI corpus lending at a 3% spread, a Department of Pharmaceuticals scheme that reimburses animal-study and clinical-investigation cost, a 20% capital subsidy written for exactly the plant you are building, and a Tamil Nadu package that expires in March 2027. Most of it is invisible from inside a company and none of it is on the JD's critical path. It should be.

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Method: the job description, floaid.com, CDSCO notified-device schedules, EEPC India sector data, and primary scheme documents from ANRF, BIRAC, TDB, the Department of Pharmaceuticals, SIDBI, NCGTC and the Government of Tamil Nadu, plus 2026 funding reporting. Figures I could not verify are flagged as such throughout rather than smoothed over.